



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مني عزت غازي همام
المحافظة : الجيزة
الحى / المدينة : كرداسه
العنوان : =
رقم البطاقة : 28907012103426 رقم التليفون : 01062751807
العمر (تاريخ الميلاد) : 01-07-1989 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 09-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: منى عزت غازي همام National ID: 28907012103426	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



الداخلية

٢٠١٢

٢٠١٢



منو
عزت غزى همام حيدر
٧٨ ش السوق القديم
مركز كرداسة - الجيزة

٢٨٩٠٧٠١٢١٠٣٤٢٦

KY1426511



٢٨٩٠٧٠١٢١٠٣٤٧٦ ٦٠٢٤/١٢

يدول عمل



انثى مسمية متزوجة

صابر فاروق محمد خير عثمان

البطاقة مارية حتى ٠٣١/١٢/٢٨



Name:	منى عزت غازی همام	
MR. :	10299	
Detailed Medical Report		
		السيد
تخية طية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه A 36-year-old patient with left-sided large-breast carcinoma, triple-negative breast cancer (TNBC), was started on neoadjuvant chemotherapy (NACT) with Taxol + Carboplatin week 1 for a total of 12 weeks followed by AC for 4 cycles. Keytruda was not yet approved; the patient received a week 3 taxane-carboplatin (TC) regimen with solucortef and avil for a recurrent hypersensitivity reaction. For week 4 TC, labs were pending and Keytruda approval remained pending. Day-care pharmacy noted a 7-fold elevation of liver enzymes and advised liver support with repeat liver function tests next week; the patient weighed 76 kg, height 167 cm, BSA 1.8 m², adjusted body weight 65.6 kg. The patient received week 4 Taxol 140 mg + Carboplatin AUC 2. The patient was scheduled for NACT TC for 12 weeks; week 5 TC, week 6 TC, pharmacy 2/7, week 7 TC pharmacy with new labs on 9 July, week 8 TC clinic on 16 July, and week 9 TC on 23 July. On 9 July, the patient received week 7 Taxol 140 mg + Carboplatin AUC 2 + GCSF; labs showed TLC/ANC 3/1.39. On 16 July, the patient did not receive GCSF; week 8 Taxol 140 mg + Carboplatin AUC 2 + GCSF; labs showed TLC/ANC 3/1.22. On 23 July, the patient was scheduled for week 9 Taxol Carboplatin; CBC from 2 weeks prior showed ANC 1.22, and a new CBC was pending for cycle continuation. On 30 July, the patient was still on week 9 Taxol Carboplatin with labs pending for weeks 10 and 11. On 6 August, the patient received week 10 Taxol 140 mg + Carboplatin AUC 2, with weight 76 kg, height 167 cm, BSA 1.8 m², adjusted body weight 65.6 kg. On 13 August, the patient received week 11 Taxol 140 mg + Carboplatin AUC 2 with liver support added. On 20 August, the patient was scheduled for week 12 Taxol Carboplatin AUC 2 and to start Doxorubicin-Cyclophosphamide (AC) with GCSF coverage from day 3 to day 5 on the next visit.		
تحريرا في ,		
	الوقت :	التاريخ : 21-09-2026
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :
يعتمد ,		

The patient has completed 12 weeks of the taxane-carboplatin regimen.

د. هنادي
روني
رئيس قسم



الجمهورية العربية السورية
لصحة المرأة المصرية

05/03/2018

MDT FORM

Name:	اسم المريض	MRN:	10299
Age:	37 Y	ID:	28907812163429
Menopausal Sta:	premenopausal	Phone:	01062791057
NPI SCORE:		Marital STATUS:	married
BMI:		Family History:	+ve
CO Morbidity:	+ve	Reason of MDT:	New case for NACT

Clinical Data:

BX:	IDC GII-III	R	L
ER:	0/1	PR:	0/0
HER2:	+ve	KI67:	

Sonomammography:

Mets Workup:

History:

Newly diagnosed Lt breast IDC, cT4b N2, TNM...diagnosed during lactation - (stopped lactation in 1/2016). Q/a...diffuse Lt breast swelling & peau d'orange - Lt AxLN largest 2x2 cm

Recommendations:

1. Bone scan 2. CT CAP 3. Echo 4. BACA testing MDT to start NACT "Keynote 522 protocol" accordingly

MDT Outcome & Plan

FOR needj taxel carbo weekly 12 weeks then AC 4 cycles with keytruda 0 cycles then MRM
brca testing



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
DR HANAN EZZAT	DR AHMED NAGEIB				



PATHOLOGY REPORT



Name: منى عزت غازي همام

Patient's I.D : 28907012103426

Age: 36 Y.

Pathology Number: 424/2025

Specimen Received Data: 16 / 11 /2025

Date Report Issued: 30/ 11 /2025

Clinical data: Left breast lesion (the patient is lactating).

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed mammary tissue cores infiltrated by a malignant tumor formed of groups, nests and few tubules of moderately to markedly anaplastic ductal epithelial cells. Surrounding desmoplastic stroma with few TILs was seen. Focal necrosis was also seen. The surrounding mammary tissue showed secretory lactational changes.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone, Her-2/neu AND Ki-67 (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed negative nuclear staining of tumor cells by chromogen.

Examination of section treated for progesterone receptors revealed negative nuclear staining of tumor cells by chromogen.

Her-2/neu: moderate complete membrane staining in >10% of tumor cells (score 2+). By FISH: The HER2 NEU/ CEP 17 ratio was 0.8 & HER2 copies were 1.6.

Examination of section treated for Ki67 revealed positive nuclear stain in average of ±60-70% of tumor cells.

DIAGNOSIS: Left Breast Lesion, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade II-III.
- Negative For Estrogen Receptors, Score 0/8.
- Negative For Progesterone Receptors, Score 0/8.
- Equivocal For Her-2/neu Expression (Score 2+), Not Amplified by FISH (Her-2 Low).
- ±60-70% Ki67 Proliferative Index.

COMMENT: "In the DESTINY-Breast 04 and 06 trials, "HER2 low" was considered IHC Score 1+ or 2+/ISH negative, and "HER2 ultralow" was HER2 IHC Score of 0 (pattern 0+) with membrane staining that is incomplete and faint/barely perceptible in less than or equal to 10% of tumor cells. Breast cancers with these staining patterns may be eligible for treatment with trastuzumab-deruxtecan in the metastatic setting (but those with no staining, IHC 0, are currently excluded)."

Dr. Miran Atif

Dr. Sara Abou-Fandoud

Miran

Sara

Contrast Enhanced Mammogram study with complementary US:**❖ *Lactating lady with reported left breast invasive duct carcinoma.***

- Extremely dense breasts (BIRADS d).
- Bilateral marked background parenchymal enhancement.
- Left breast upper outer heterogeneous mass lesion with indistinct margins is seen merging with extensive non-mass enhancement seen involving/infiltrating most of breast parenchyma measuring collectively about 10.5x9cm along its maximum dimensions (measured in MLO view). Shortest distance to nipple is 2.6cm and shortest distance to skin is 1.7cm.
- Multiple pathological left axillary lymph nodes are seen at level I, II and III with the largest seen at level I measuring about 3.4x1.2 cm along its maximum dimensions.
- Right non-specific axillary lymph nodes.
- No supra-clavicular lymph nodes.

Conclusion:

- Left breast locally advanced malignant mass and non-mass enhancement with ipsilateral pathological axillary level I, II and III lymph nodes (BIRADS 6).
- Right lactational mammary changes with no pathological enhancement (BIRADS 2).

Thanks for referral,

Dr.Eman Badawy,MD

Ass.Prof Dr.Sherihan Fakhry,MD

WOMEN IMAGING DEPARTMENT

Patient Name: Mona Ezzat Ghazy
Date: 16- 11-2025

Bilateral High resolution Ultrasound Examination

Indication: lactating lady with left breast painful lump.

Findings:

- Prominent fibroglandular breast parenchyma showing prominent lactiferous ducts.
- An ill-defined irregular shape hypoechoic area is seen at left breast 2 o'clock measuring about 5.6x3.2 cm along its maximum dimensions showing increased vascularity and surrounded by echogenic fat lobules.
- Normal contour and skin thickening of both breasts.
- Normal nipple /areola complexes.
- Right nonspecific axillary lymph nodes with preserved fatty hilum.
- Multiple enlarged left axillary level I, II and III lymph nodes with the largest seen at level I measuring about 2.7x1.5 cm showing diffuse cortical thickening and compressed hilum.

Opinion :-

- Left breast lactational changes in addition to 2 o'clock ill defined irregular shaped hypoechoic area with increased vascularity and surrounding echogenic fat with ipsilateral axillary lymphadenopathy, likely inflammatory process/phlegmon, granulomatous mastitis is to be considered (BIRADS 3), for histopathology.
- Right breast lactational changes (BIRADS 2).

Thanks for referral,
Dr. Eman Badawy, MD

1.999

Name: منى عزت غازى			26/4/2026		
LVEDD	5	(3.5-5.6 cm)	Left Atrium	3	(1.9-4.0 cm)
LVESD	3.6	(2.5 -4 cm)	Aortic root	2.6	(2.0-3.7 cm)
LVSWT	.9	(0.7-1 cm)	AO.Exeursion		(1.6-2.6 cm)
LVPWT	.98	(0.7-1 c m)	RT.Ventricle		(0.7-2.3 cm)
EF	65	50-65 %	F S		25-40 %

Comment:

- Normal left ventricle internal dimensions with good systolic function
- No resting wall motion abnormalities at rest study Coronary artery disease couldn't be
- Normal cardiac valvas structure and mobility
- Normal intra-cardiac chambers dimensions
- No intra-caraic mass or thromi
- pulmonary Artery systolic pressure 32 mmHg

Conclusion:

- Good left systolic performance EF =65%
- No Resting wall motion abnormmalies
- Normal cardiac valvas structure and mobility



Patient Name:	Mona Ezat Ghazy Hamam	Patient Gender:	Female
Patient Code:	Q49073	DATE:	20-Apr-26 10:58:40 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- A female patient with history of left breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Osteoarthritic changes of the both knees are seen.

Opinion

- No scintigraphic evidence of metastatic osseous lesions.

Dr. Alaa Sayed

Prof. Hosna Moustafa, MD.